PharmEasy Labs

Report for Kamran Mulla(36Y/M)

Tests asked Ek In 109 03Hc Revised

Test date 20 Oct 2024 **Report status** Complete Report

REPORT VERIFIED VERIFIED BY DOCTOR

6 STEP quality control to ensure 100% report accuracy

- Qualified and trained technicians
- Temperature-controlled containers to store samples
- Strict quality checks on samples before processing
- Regular monitoring of lab analyzers by experts
- Assured machine inspection on a daily basis
- Verified reports by qualified pathologists

25+ Years of Trust & Experience | NABL Accredited Labs | 100+ Crore Samples Processed

PharmEasy Labs

Name : KAMRAN MULLA(36Y/M)

Ref. By : SELF

ADDRESS :

FLAT 402 B WING DYANDHARSHAN BUILDING DYANDHARSHAN BUILDING MUMBAI MAHARASHTRA INDIA 400067

Report Availability Summary

Full Report Available

Note : This is summary page. Please refer to the table below for the details

Test	Report Status
25-OH VITAMIN D (TOTAL)	Available
BLOOD GROUPING AND RH TYPING	Available
BLOOD GROUPING	Available
RH TYPING	Available
COMPLETE URINE ANALYSIS	Available
ERYTHROCYTE SEDIMENTATION RATE (ESR)	Available
FASTING BLOOD SUGAR(GLUCOSE)	Available
FERRITIN	Available
HbA1c	Available
HEMOGRAM - 6 PART (DIFF)	Available
IRON DEFICIENCY PROFILE	Available
IRON	Available
TOTAL IRON BINDING CAPACITY (TIBC)	Available
UNSAT.IRON-BINDING CAPACITY(UIBC)	Available
KIDPRO	Available
LIPID PROFILE	Available
LIVER FUNCTION TESTS	Available
T3-T4-TSH	Available
VITAMIN B-12	Available

Note : Underlined values are Critical Values, Clinician's attention required.

Clinically Tested by : Thyrocare Technologies Ltd.

PharmEasy Labs

NAME : KAMRAN MULLA(36Y/M)
REF. BY : SELF
TEST ASKED : EK IN 109 03HC REVISED

HOME COLLECTION :
FLAT 402 B WING DYANDHARSHAN BUILDING
DYANDHARSHAN BUILDING MUMBAI
MAHARASHTRA INDIA 400067

TEST NAME	TECHNOLOGY	VALUE	UNITS
HbA1c - (HPLC)			
	H.P.L.C	**9.3**	**%**

Bio. Ref. Interval. :

Bio. Ref. Interval.: As per ADA Guidelines	Guidance For Known Diabetics
Below 5.7% : Normal	Below 6.5% : Good Control
5.7% - 6.4% : Prediabetic	6.5% - 7% : Fair Control
>=6.5% : Diabetic	7.0% - 8% : Unsatisfactory Control
	>8% : Poor Control

Method : Fully Automated H.P.L.C method

AVERAGE BLOOD GLUCOSE (ABG)	**CALCULATED**	**220**	**mg/dL**

Bio. Ref. Interval. :

90 - 120 mg/dl : Good Control
121 - 150 mg/dl : Fair Control
151 - 180 mg/dl : Unsatisfactory Control
> 180 mg/dl : Poor Control

Method : Derived from HBA1c values

Please correlate with clinical conditions.

Sample Collected on (SCT) : 20 Oct 2024 08:25
Sample Received on (SRT) : 20 Oct 2024 16:40
Report Released on (RRT) : 20 Oct 2024 21:19
Sample Type : EDTA Whole Blood
Labcode : 2010001452/DG126
Barcode : DA174943

Dr Renuka MD(Path)

Dr Arshiya MD(Path)

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Clinically Tested by :Thyrocare Technologies Ltd - (NGSP accredited)

PharmEasy Labs
Registered Address: DOCON TECHNOLOGIES PRIVATE LIMITED, Office No 208, 209 & 210 Second floor, A wing, 'Dattani Plaza', Near East West Industrial Estate, Safed Pool, Saki Naka, Andheri (East), Mumbai - 400072
Lab Address: D-37/1. TTC MIDC, Turbhe. Navi Mumbai, 400703
7022 000 900

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NAME	: KAMRAN MULLA(36Y/M)	**HOME COLLECTION :**	
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TEST ASKED	: EK IN 109 03HC REVISED		

TEST NAME	TECHNOLOGY	VALUE	UNITS
ERYTHROCYTE SEDIMENTATION RATE (ESR)	**MODIFIED WESTERGREN**	**94**	**mm / hr**
Bio. Ref. Interval. :-			
Male : 0-15 Female : 0-20			

Please correlate with clinical conditions.

Method:- MODIFIED WESTERGREN

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TEST ASKED	: EK IN 109 03HC REVISED		

TEST NAME	TECHNOLOGY	VALUE
BLOOD GROUPING	AGGLUTINATION	A
RH TYPING	AGGLUTINATION	POSITIVE

Method:- Processed on fully automated Matrix Automax analyser based on gel column agglutination technology.

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MAHARASHTRA INDIA 400067

TEST NAME	METHODOLOGY	VALUE	UNITS	Bio. Ref. Interval.
HEMOGLOBIN	**SLS-Hemoglobin Method**	**12.5**	**g/dL**	**13.0-17.0**
Hematocrit (PCV)	**CPH Detection**	**39.6**	**%**	**40.0-50.0**
Total RBC	HF & EI	4.71	X 10^6/µL	4.5-5.5
Mean Corpuscular Volume (MCV)	Calculated	84.1	fL	83.0-101.0
Mean Corpuscular Hemoglobin (MCH)	**Calculated**	**26.5**	**pq**	**27.0-32.0**
Mean Corp.Hemo. Conc (MCHC)	Calculated	31.6	g/dL	31.5-34.5
Red Cell Distribution Width - SD (RDW-SD)	Calculated	40.9	fL	39-46
Red Cell Distribution Width (RDW - CV)	Calculated	13.2	%	11.6-14
TOTAL LEUCOCYTE COUNT (WBC)	HF & FC	7.48	X 10^3 / µL	4.0 - 10.0
DIFFERENTIAL LEUCOCYTE COUNT				
Neutrophils Percentage	Flow Cytometry	60	%	40-80
Lymphocytes Percentage	Flow Cytometry	32.9	%	20-40
Monocytes Percentage	Flow Cytometry	2.9	%	2-10
Eosinophils Percentage	Flow Cytometry	3.1	%	1-6
Basophils Percentage	Flow Cytometry	0.8	%	0-2
Immature Granulocyte Percentage (IG%)	Flow Cytometry	0.3	%	0-0.5
Nucleated Red Blood Cells %	Flow Cytometry	0.01	%	0.0-5.0
ABSOLUTE LEUCOCYTE COUNT				
Neutrophils - Absolute Count	Calculated	4.49	X 10^3 / µL	2.0-7.0
Lymphocytes - Absolute Count	Calculated	2.46	X 10^3 / µL	1.0-3.0
Monocytes - Absolute Count	Calculated	0.22	X 10^3 / µL	0.2 - 1.0
Basophils - Absolute Count	Calculated	0.06	X 10^3 / µL	0.02 - 0.1
Eosinophils - Absolute Count	Calculated	0.23	X 10^3 / µL	0.02 - 0.5
Immature Granulocytes (IG)	Calculated	0.02	X 10^3 / µL	0-0.3
Nucleated Red Blood Cells	Calculated	0.01	X 10^3 / µL	0.0-0.5
PLATELET COUNT	HF & EI	386	X 10^3 / µL	150-410
Mean Platelet Volume (MPV)	Calculated	10	fL	6.5-12
Platelet Distribution Width (PDW)	Calculated	11.7	fL	9.6-15.2
Platelet to Large Cell Ratio (PLCR)	Calculated	25.5	%	19.7-42.4
Plateletcrit (PCT)	Calculated	0.39	%	0.19-0.39

Remarks : Alert!!! Predominantly normocytic normochromic with ovalocytes. Platelets:Appear adequate in smear.

Clinical history is asked for all the relevant abnormalities detected and in absence / failure of receiving of clinical history, results are rechecked twice and released. Advised clinical correlation.

Method : Fully automated bidirectional analyser (6 Part Differential SYSMEX XN-1000)
(Reference : *FC- flowcytometry, *HF- hydrodynamic focussing, *EI- Electric Impedence, *Hb- hemoglobin, *CPH- Cumulative pulse height**)**

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TEST ASKED	: EK IN 109 03HC REVISED		

TEST NAME	TECHNOLOGY	VALUE	UNITS
FASTING BLOOD SUGAR(GLUCOSE)	**PHOTOMETRY**	**113.69**	**mg/dL**

Bio. Ref. Interval. :-

As per ADA Guideline: Fasting Plasma Glucose (FPG)	
Normal	70 to 100 mg/dl
Prediabetes	100 mg/dl to 125 mg/dl
Diabetes	126 mg/dl or higher

Note :
The assay could be affected mildly and may result in anomalous values if serum samples have heterophilic antibodies, hemolyzed , icteric or lipemic. The concentration of Glucose in a given specimen may vary due to differences in assay methods, calibration and reagent specificity. For diagnostic purposes results should always be assessed in conjunction with patients medical history, clinical findings and other findings.

Please correlate with clinical conditions.

Method:- GOD-PAP METHOD

Sample Collected on (SCT)	: 20 Oct 2024 08:25
Sample Received on (SRT)	: 20 Oct 2024 10:18
Report Released on (RRT)	: 20 Oct 2024 11:27
Sample Type	: FLUORIDE PLASMA
Labcode	: 2010065114/DG126
Barcode	: DD130352

Dr Vitthal Kendre MD(Path)

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TEST ASKED	: EK IN 109 03HC REVISED		

TEST NAME	TECHNOLOGY	VALUE	UNITS
25-OH VITAMIN D (TOTAL)	**E.C.L.I.A**	**8.11**	**ng/mL**

Bio. Ref. Interval. :-

Deficiency : <=20 ng/ml || Insufficiency : 21-29 ng/ml
Sufficiency : >= 30 ng/ml || Toxicity : >100 ng/ml

Clinical Significance:
Vitamin D is a fat soluble vitamin that has been known to help the body absorb and retain calcium and phosphorous; both are critical for building bone health.
Decrease in vitamin D total levels indicate inadequate exposure of sunlight, dietary deficiency, nephrotic syndrome.
Increase in vitamin D total levels indicate Vitamin D intoxication.

Specifications: Precision: Intra assay (%CV):9.20%, Inter assay (%CV):8.50%
Kit Validation Reference : Holick M. Vtamin D the underappreciated D-Lightful hormone that is important for Skeletal and cellular health Curr Opin Endocrinol Diabetes 2002:9(1)87-98.

Please correlate with clinical conditions.

Method:- Fully Automated Electrochemiluminescence Compititive Immunoassay

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Sample Type	: SERUM
Labcode	: 2010089596/DG126
Barcode	: CZ048441

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Dr Arshiya MD(Path)

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MAHARASHTRA INDIA 400067

TEST NAME	TECHNOLOGY	VALUE	UNITS
IRON	PHOTOMETRY	86	µg/dL
Bio. Ref. Interval. : Male : 65 - 175 Female : 50 - 170 **Method :** Ferrozine method without deproteinization			
TOTAL IRON BINDING CAPACITY (TIBC)	**PHOTOMETRY**	**544**	**µg/dL**
Bio. Ref. Interval. : Male: 225 - 535 µg/dl Female: 215 - 535 µg/dl **Method :** Spectrophotometric Assay			
% TRANSFERRIN SATURATION	CALCULATED	16	%
Bio. Ref. Interval. : 13 - 45 **Method :** Derived from IRON and TIBC values			
FERRITIN	E.C.L.I.A	400	ng/mL
Bio. Ref. Interval. : 30 - 400 **Method :** Fully Automated Electrochemiluminescence Sandwich Immunoassay			
UNSAT.IRON-BINDING CAPACITY(UIBC)	**PHOTOMETRY**	**457.5**	**µg/dL**
Bio. Ref. Interval. : 162 - 368 **Method :** SPECTROPHOTOMETRIC ASSAY			

Please correlate with clinical conditions.

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TEST ASKED	: EK IN 109 03HC REVISED		

TEST NAME	TECHNOLOGY	VALUE	UNITS
VITAMIN B-12	E.C.L.I.A	222	pg/mL

Bio. Ref. Interval. :-

Normal: 197-771 pg/ml

Clinical significance :
Vitamin B12 or cyanocobalamin, is a complex corrinoid compound found exclusively from animal dietary sources, such as meat, eggs and milk. It is critical in normal DNA synthesis, which in turn affects erythrocyte maturation and in the formation of myelin sheath. Vitamin-B12 is used to find out neurological abnormalities and impaired DNA synthesis associated with macrocytic anemias. For diagnostic purpose, results should always be assessed in conjunction with the patients medical history, clinical examination and other findings.

Specifications: Intra assay (%CV):2.6%, Inter assay (%CV):2.3 %

Kit Validation Reference : Thomas L.Clinical laborator Diagnostics : Use and Assessment of Clinical laboratory Results 1st Edition,TH Books-Verl-Ges,1998:424-431

Please correlate with clinical conditions.

Method:- Fully Automated Electrochemiluminescence Compititive Immunoassay

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INDIA 400067

TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
TOTAL CHOLESTEROL	PHOTOMETRY	201	mg/dL	< 200
HDL CHOLESTEROL - DIRECT	PHOTOMETRY	20	mg/dL	40-60
LDL CHOLESTEROL - DIRECT	PHOTOMETRY	133	mg/dL	< 100
TRIGLYCERIDES	PHOTOMETRY	290	mg/dL	< 150
TC/ HDL CHOLESTEROL RATIO	CALCULATED	9.9	Ratio	3 - 5
TRIG / HDL RATIO	CALCULATED	14.29	Ratio	< 3.12
LDL / HDL RATIO	CALCULATED	6.6	Ratio	1.5-3.5
HDL / LDL RATIO	CALCULATED	0.15	Ratio	> 0.40
NON-HDL CHOLESTEROL	CALCULATED	180.3	mg/dL	< 160
VLDL CHOLESTEROL	CALCULATED	58	mg/dL	5 - 40

Please correlate with clinical conditions.

Method :

CHOL - Cholesterol Oxidase, Esterase, Peroxidase
HCHO - Direct Enzymatic Colorimetric
LDL - Direct Measure
TRIG - Enzymatic, End Point
TC/H - Derived from serum Cholesterol and Hdl values
TRI/H - Derived from TRIG and HDL Values
LDL/ - Derived from serum HDL and LDL Values
HD/LD - Derived from HDL and LDL values.
NHDL - Derived from serum Cholesterol and HDL values
VLDL - Derived from serum Triglyceride values

***REFERENCE RANGES AS PER NCEP ATP III GUIDELINES:**

TOTAL CHOLESTEROL	(mg/dl)	HDL	(mg/dl)	LDL	(mg/dl)	TRIGLYCERIDES	(mg/dl)
DESIRABLE	<200	LOW	<40	OPTIMAL	<100	NORMAL	<150
BORDERLINE HIGH	200-239	HIGH	>60	NEAR OPTIMAL	100-129	BORDERLINE HIGH	150-199
HIGH	>240			BORDERLINE HIGH	130-159	HIGH	200-499
				HIGH	160-189	VERY HIGH	>500
				VERY HIGH	>190		

Alert !!! 10-12 hours fasting is mandatory for lipid parameters. If not, values might fluctuate.

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TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
ALKALINE PHOSPHATASE	**PHOTOMETRY**	**33.3**	**U/L**	**45-129**
BILIRUBIN - TOTAL	PHOTOMETRY	0.48	mg/dL	0.3-1.2
BILIRUBIN -DIRECT	PHOTOMETRY	0.12	mg/dL	< 0.3
BILIRUBIN (INDIRECT)	CALCULATED	0.36	mg/dL	0-0.9
GAMMA GLUTAMYL TRANSFERASE (GGT)	PHOTOMETRY	52.8	U/L	< 55
ASPARTATE AMINOTRANSFERASE (SGOT)	PHOTOMETRY	19.7	U/L	< 35
ALANINE TRANSAMINASE (SGPT)	PHOTOMETRY	15.1	U/L	< 45
SGOT / SGPT RATIO	CALCULATED	1.3	Ratio	< 2
PROTEIN - TOTAL	PHOTOMETRY	7.65	gm/dL	5.7-8.2
ALBUMIN - SERUM	PHOTOMETRY	4.31	gm/dL	3.2-4.8
SERUM GLOBULIN	CALCULATED	3.34	gm/dL	2.5-3.4
SERUM ALB/GLOBULIN RATIO	CALCULATED	1.29	Ratio	0.9 - 2

Please correlate with clinical conditions.

Method :

ALKP - Modified IFCC method
BILT - Vanadate Oxidation
BILD - Vanadate Oxidation
BILI - Derived from serum Total and Direct Bilirubin values
GGT - Modified IFCC method
SGOT - IFCC* Without Pyridoxal Phosphate Activation
SGPT - IFCC* Without Pyridoxal Phosphate Activation
OT/PT - Derived from SGOT and SGPT values.
PROT - Biuret Method
SALB - Albumin Bcg¹method (Colorimetric Assay Endpoint)
SEGB - DERIVED FROM SERUM ALBUMIN AND PROTEIN VALUES
A/GR - Derived from serum Albumin and Protein values

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TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
BLOOD UREA NITROGEN (BUN)	PHOTOMETRY	19.96	mg/dL	7.94 - 20.07
CREATININE - SERUM	**PHOTOMETRY**	**1.33**	**mg/dL**	**0.72-1.18**
BUN / SR.CREATININE RATIO	CALCULATED	15.01	Ratio	9:1-23:1
UREA (CALCULATED)	CALCULATED	42.71	mg/dL	Adult : 17-43
UREA / SR.CREATININE RATIO	CALCULATED	32.12	Ratio	< 52
CALCIUM	PHOTOMETRY	9.76	mg/dL	8.8-10.6
URIC ACID	**PHOTOMETRY**	**7.4**	**mg/dL**	**4.2 - 7.3**

Please correlate with clinical conditions.

Method :

BUN - Kinetic UV Assay.
SCRE - Creatinine Enzymatic Method
B/CR - Derived from serum Bun and Creatinine values
UREAC - Derived from BUN Value.
UR/CR - Derived from UREA and Sr.Creatinine values.
CALC - Arsenazo III Method, End Point.
URIC - Uricase / Peroxidase Method

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TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
TOTAL TRIIODOTHYRONINE (T3)	C.L.I.A	80	ng/dL	60-200
TOTAL THYROXINE (T4)	C.L.I.A	9.9	µg/dL	4.5-12
THYROID STIMULATING HORMONE (TSH)	C.L.I.A	1.80	µIU/mL	0.3-5.5

Comments : SUGGESTING THYRONORMALCY

The Biological Reference Ranges is specific to the age group. Kindly correlate clinically.

Method :

T3 - Competitive Chemi Luminescent Immuno Assay
T4 - Competitive Chemi Luminescent Immuno Assay
TSH - Sandwich Chemi Luminescent Immuno Assay

Disclaimer :

Results should always be interpreted using the reference range provided by the laboratory that performed the test.
Different laboratories do tests using different technologies, methods and using different reagents which may cause difference
In reference ranges and hence it is recommended to interpret result with assay specific reference ranges provided in the reports.
To diagnose and monitor therapy doses, it is recommended to get tested every time at the same Laboratory.

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TEST ASKED	: EK IN 109 03HC REVISED		

TEST NAME	TECHNOLOGY	VALUE	UNITS
EST. GLOMERULAR FILTRATION RATE (eGFR)	**CALCULATED**	**68**	**mL/min/1.73 m2**

Bio. Ref. Interval. :-

> = 90 : Normal
60 - 89 : Mild Decrease
45 - 59 : Mild to Moderate Decrease
30 - 44 : Moderate to Severe Decrease
15 - 29 : Severe Decrease

Clinical Significance

The normal serum creatinine reference interval does not necessarily reflect a normal GFR for a patient. Because mild and moderate kidney injury is poorly inferred from serum creatinine alone. Thus, it is recommended for clinical laboratories to routinely estimate glomerular filtration rate (eGFR), a "gold standard" measurement for assessment of renal function, and report the value when serum creatinine is measured for patients 18 and older, when appropriate and feasible. It cannot be measured easily in clinical practice, instead, GFR is estimated from equations using serum creatinine, age, race and sex. This provides easy to interpret information for the doctor and patient on the degree of renal impairment since it approximately equates to the percentage of kidney function remaining. Application of CKD-EPI equation together with the other diagnostic tools in renal medicine will further improve the detection and management of patients with CKD.

Reference

Levey AS, Stevens LA, Schmid CH, Zhang YL, Castro AF, 3rd, Feldman HI, et al. A new equation to estimate glomerular filtration rate. Ann Intern Med. 2009;150(9):604-12.

Please correlate with clinical conditions.

Method:- CKD-EPI Creatinine Equation

Sample Collected on (SCT)	: 20 Oct 2024 08:25
Sample Received on (SRT)	: 20 Oct 2024 16:15
Report Released on (RRT)	: 20 Oct 2024 19:19
Sample Type	: SERUM
Labcode	: 2010089596/DG126
Barcode	: CZ048441

Dr Renuka MD(Path)

Dr Arshiya MD(Path)

Note:- Underlined values are Critical Values, Clinician's attention required.

Clinically Tested by :Thyrocare Technologies Ltd - (NABL accredited)

PharmEasy Labs

NAME : KAMRAN MULLA(36Y/M)
REF. BY : SELF
TEST ASKED : EK IN 109 03HC REVISED

HOME COLLECTION :
FLAT 402 B WING DYANDHARSHAN BUILDING DYANDHARSHAN BUILDING MUMBAI MAHARASHTRA INDIA 400067

TEST NAME	METHODOLOGY	VALUE	UNITS	Bio. Ref. Interval.
Complete Urinogram				
Physical Examination				
VOLUME	Visual Determination	3	mL	-
COLOUR	Visual Determination	PALE YELLOW	-	Pale Yellow
APPEARANCE	Visual Determination	CLEAR	-	Clear
SPECIFIC GRAVITY	pKa change	1.025	-	1.003-1.030
PH	pH indicator	5	-	5-8
Chemical Examination				
URINARY PROTEIN	PEI	ABSENT	mg/dL	Absent
URINARY GLUCOSE	**GOD-POD**	**Present 3+(500-1000 mg/dl)**	**mg/dL**	**Absent**
URINE KETONE	Nitroprusside	ABSENT	mg/dL	Absent
URINARY BILIRUBIN	Diazo coupling	ABSENT	mg/dL	Absent
UROBILINOGEN	Diazo coupling	Normal	mg/dL	<=0.2
BILE SALT	Hays sulphur	ABSENT	-	Absent
BILE PIGMENT	Ehrlich reaction	ABSENT	-	Absent
URINE BLOOD	Peroxidase reaction	ABSENT	-	Absent
NITRITE	Diazo coupling	ABSENT	-	Absent
LEUCOCYTE ESTERASE	Esterase reaction	ABSENT	-	Absent
Microscopic Examination				
MUCUS	Microscopy	ABSENT	-	Absent
RED BLOOD CELLS	Microscopy	ABSENT	cells/HPF	0-5
URINARY LEUCOCYTES (PUS CELLS)	Microscopy	ABSENT	cells/HPF	0-5
EPITHELIAL CELLS	Microscopy	ABSENT	cells/HPF	0-5
CASTS	Microscopy	ABSENT	-	Absent
CRYSTALS	Microscopy	ABSENT	-	Absent
BACTERIA	Microscopy	ABSENT	-	Absent
YEAST	Microscopy	ABSENT	-	Absent
PARASITE	Microscopy	ABSENT	-	Absent

(Reference : *PEI - Protein error of indicator, *GOD-POD - Glucose oxidase-peroxidase**)**

~~ End of report ~~

Sample Collected on (SCT) : 20 Oct 2024 08:25
Sample Received on (SRT) : 20 Oct 2024 11:01
Report Released on (RRT) : 20 Oct 2024 12:14
Sample Type : URINE
Labcode : 2010066497/DG126
Barcode : CZ231943

Dr Vitthal Kendre MD(Path)

Note:- Underlined values are Critical Values, Clinician's attention required.

Clinically Tested by :Thyrocare Technologies Ltd

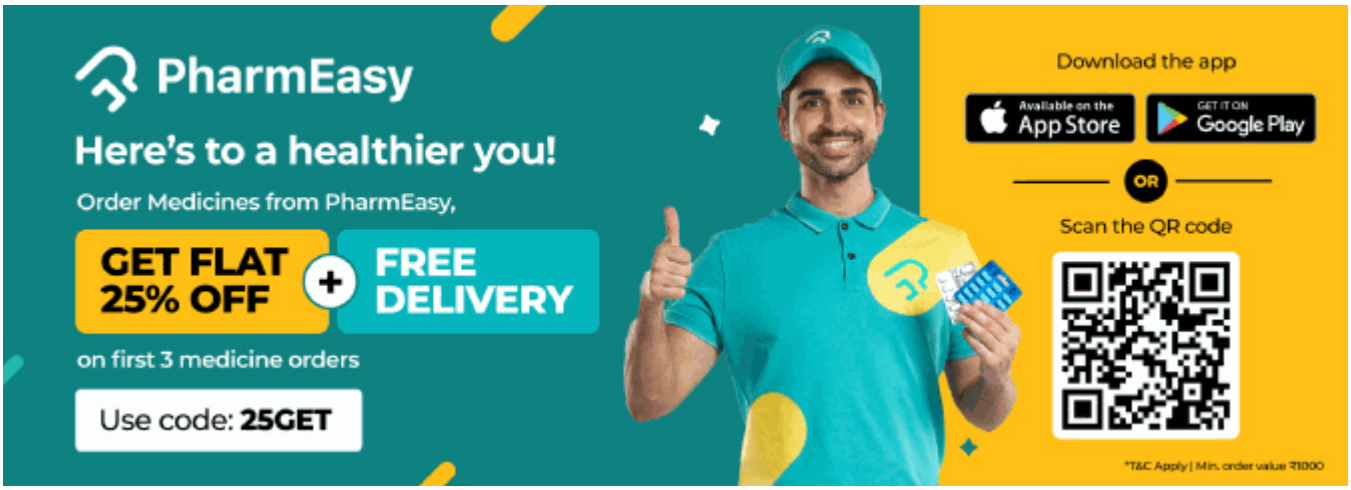
PharmEasy Labs
Registered Address: DOCON TECHNOLOGIES PRIVATE LIMITED, Office No 208, 209 & 210 Second floor, A wing, 'Dattani Plaza', Near East West Industrial Estate, Safed Pool, Saki Naka, Andheri (East), Mumbai - 400072
Lab Address: 1st floor, Avirahi Arcade, Kandivali (W), Mumbai - 400067
7022 000 900

CONDITIONS OF REPORTING

- The reported results are for information and interpretation of the referring doctor only.
- It is presumed that the tests performed on the specimen belong to the patient; named or identified.
- Results of tests may vary from laboratory to laboratory and also in some parameters from time to time for the same patient.
- Should the results indicate an unexpected abnormality, the same should be reconfirmed.
- Only such medical professionals who understand reporting units, reference ranges and limitations of technologies should interpret results.
- This report is not valid for medico-legal purpose.
- Docon Technologies Private Limited,Thyrocare Technologies Limited and its employees/representatives do not assume any liability,responsibility for any loss or damage that may be incurred by any person as a result of presuming the meaning or contents of the report.

EXPLANATIONS

- **Name -** The name is as declared by the client and recorded by the personnel who collected the specimen.
- **Ref.By -** The name of the doctor who has recommended testing as declared by the client.
- **Labcode -** This is the accession number in our laboratory and it helps us in archiving and retrieving the data.
- **Barcode -** This is the specimen identity number and it states that the results are for the specimen bearing the barcode (irrespective of the name).
- **SCT -** Specimen Collection Time - The time when specimen was collected as declared by the client.
- **SRT -** Specimen Receiving Time - This time when the specimen reached our laboratory.
- **RRT -** Report Releasing Time - The time when our pathologist has released the values for Reporting.
- **Reference Range -** Means the range of values in which 95% of the normal population would fall.

SUGGESTIONS

- Values out of reference range requires reconfirmation before starting any medical treatment.
- Retesting is needed if you suspect any quality shortcomings.
- For suggestions, complaints or feedback, write to us at grievance-office@docon.co.in or call us on 7022000900.